



Corporate Office : HY PATHO LAB

📍 CB 252, GROUND & FIRST FLOOR NARAINA, NEW DELHI 110028

☎ 7718962488

✉ info@hyatholab.in

🌐 www.hyatholab.in

☎ Toll Free No. : 18001030287

REPORT

PT Name : SAURABH KOTHARI

Age : 40 Year | Sex : Male

SAMPLE COLLECTED AT :

Ref By : DR. SELF

Registered on : 02-09-2026 02:12 PM

SHREE LAB

Reg No : HL3301832634 / HYMH/KDV/H6569

Received on : 02-09-2026 05:15 PM

Barcode : HEE125362

Reported on : 03-09-2026 12:11 AM



INV : HY FITNESS SMART 3

SAMPLE : Serum

25 OH VITAMIN D 3 TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
25-OH VITAMIN D 3 (TOTAL) Method: CLIA	28.0	ng/mL	30 - 100
Reference Range Deficiency < 20 ng/mL Insufficiency 20– 30 ng/mL Sufficiency 30–100 ng/mL Toxicity >100 ng/mL			

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.

Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~



Dr. NEHAL DATTARAM RAHATE (MD PATH)
Consultant Pathologist



Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



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REPORT

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Ref By : DR. SELF

Reg No : HL3301832634 / HYMH/KDV/H6569

Barcode : HEE125362

INV : HY FITNESS SMART 3

Age : 40 Year | Sex : Male

Registered on : 02-09-2026 02:12 PM

Received on : 02-09-2026 05:15 PM

Reported on : 03-09-2026 12:11 AM

SAMPLE COLLECTED AT :

SHREE LAB



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Reference Range Deficiency < 20 ng/mL Insufficiency 20– 30 ng/mL Sufficiency 30–100 ng/mL Toxicity >100 ng/mL			

Vitamin D Total test is analyzed on Siemens ADVIA Centaur, standardized against ID-LC/MS/MS, as per Vitamin D Standardization Program (VDSP).

Specifications: Intra assay (%CV):5.3%, Inter assay (%CV):11.9% ; Sensitivity:3.2 ng/ml

Please correlate with clinical conditions.

Method : Fully Automated Chemi Luminescent Immuno Assay

~~End of report~~



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Consultant Pathologist



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REPORT

PT Name : MRS. PRATIBHA SADANAND JOSHI
Ref By : DR. DR. AJAYSING PARDESHI SIR (M.D.)
Reg No : HL1701807513 / MH130
Barcode : HEB106819
INV : BODY FLUID FOR GRAM STAIN

Age : 55 Year | Sex : Female
Registered on : 08-08-2026 10:14 PM
Received on : 09-08-2026 02:13 PM
Reported on : 09-08-2026 11:45 PM

SAMPLE COLLECTED AT :
OM LAB PACHORA



SAMPLE : Body Fluid

BODY FLUID FOR GRAM STAIN

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TYPE OF SPECIMEN	FLUID	.	..
ORGANISM Method: MICROSCOPY	NO MICRO-ORGANISM SEEN	.	..

~~End of report~~

Pallavi.

Dr. PALLAVI SAXENA (MD PATH)
Consultant Pathologist (Reg. 0375)



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Toll Free No. : 18001030287

REPORT

PT Name : ELIYAS SHAIKH

Age : 62 Year | Sex : Male

SAMPLE COLLECTED AT :

Ref By : DR. CENTRAL HOSPITAL

Registered on : 29-08-2026 11:36 AM

NAGRIK DIAGNOSTICS

Reg No : HL4901829960 / RAJ02

Received on : 29-08-2026 06:58 PM

Barcode : HV027093

Reported on : 06-09-2026 12:17 AM



INV : FNAC (Fine Needle Aspiration Cytology)

SAMPLE : SLIDE

FNAC (FINE NEEDLE ASPIRATION CYTOLOGY)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
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FNAC SITE:

GROSS: Received Three unstained slides.

MICROSCOPY: Smears show abundant neutrophils and polymorphous population of lymphoid cells in a background of extensive necrotic debris and proteinaceous material. No malignant cells are identified in the examined smears.

IMPRESSION: Features are consistent with an acute suppurative inflammatory lesion (lymph node)

Advise: Clinicoradiological correlation and microbiological culture are advised.

~~End of report~~

NRahate

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Toll Free No. : 18001030287

REPORT

PT Name : SHRIRANG SANGORAM

Ref By : DR. SELF

Reg No : HL4901832195 / MH026

Barcode : 396709455

INV : IMMUNOGLOBULIN-IgE(Total IgE)

Age : 54 Year | Sex : Male

Registered on : 31-08-2026 02:59 PM

Received on : 01-09-2026 04:44 PM

Reported on : 01-09-2026 09:12 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

IMMUNOGLOBULIN-IGE(TOTAL IGE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL IgE Method: CLIA	3000	IU/mL	00 - 200

Reference Range

Age	Value
0 - 1 Year	1.40 - 52.3
1 - 4 Years	0.40 - 351.6
5 - 10 Years	0.50 - 393
11 - 15 Years	1.90 - 170
Adults	< 200

NOTE-

Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.

No close correlation has been demonstrated between severity of allergic reaction and IgE levels.

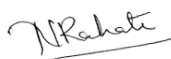
Comments

Immunoglobulin E (IgE) is the most important trigger molecule for allergic information. The level of IgE is low during the first year of life, gradually increases with age and reaches adult levels after 10 years. As IgE is a mediator of allergic response, quantitative measurement can provide useful information for differential diagnosis of atopic and non-atopic disease. Patients with atopic diseases like Allergic asthma, Allergic rhinitis & Atopic dermatitis have moderately elevated IgE levels.

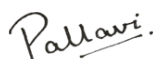
Increased Levels - Atopic/Non-atopic allergy, Hyper IgE syndrome, Parasitic infections, IgE Myeloma, Pulmonary Aspergillosis, Immunodeficiency states & Autoimmune diseases

Please correlate with clinical conditions.

~~End of report~~



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REPORT

PT Name : SHABNAM

Ref By : DR. SELF

Reg No : HL6301832464 / MH019

Barcode : Hei087746

INV : Insulin Fasting

Age : 52 Year | Sex : Female

Registered on : 31-08-2026 05:25 PM

Received on : 31-08-2026 09:21 PM

Reported on : 31-08-2026 10:06 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

INSULIN FASTING

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN FASTING	5.72	u/ml	3.0 - 25.0

Method: CLIA

Clinical significance :

1. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.
2. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines
3. Interference in insulin assay is seen due to insulin antibodies which develop in patients treated with bovine or porcine insulin.

Clinical Utility

- Evaluation of fasting hypoglycemia
- Evaluation of Polycystic Ovary syndrome
- Classification of Diabetes mellitus
- Predict Diabetes mellitus
- Assessment of Beta cell activity
- Select optimal therapy for Diabetes
- Investigation of insulin resistance
- Predict the development of Coronary Artery Disease

Increased Levels – Insulinoma, Some Type II diabetic patients, Infantile hypoglycemia, Hyperinsulinism,

Obesity, Cushing's syndrome, Oral contraceptives, Acromegaly, Hyperthyroidism

Decreased Levels - Untreated Type I Diabetes mellitus

~~End of report~~



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Toll Free No. : 18001030287

REPORT

PT Name : MONIKA GHODSE

Age : 61 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. SELF

Registered on : 29-08-2026 06:55 PM

Reg No : HL5001830586 / RAJ53

Received on : 29-08-2026 07:05 PM

Barcode : HEN143402

Reported on : 29-08-2026 09:01 PM



INV : Insulin - Post Prandial

SAMPLE : Serum

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

~~End of report~~

NRahate

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REPORT

PT Name : MONIKA GHODSE

Age : 61 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. SELF

Registered on : 29-08-2026 06:55 PM

Reg No : HL5001830586 / RAJ53

Received on : 29-08-2026 07:05 PM

Barcode : HEN143402

Reported on : 29-08-2026 09:01 PM



INV : Insulin - Post Prandial

SAMPLE : Serum

INSULIN - POST PRANDIAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
INSULIN POSTPRANDIAL (PP) Method: CLIA	15.90	U/ML	. - . Insulin 30 min : 20 – 112 Insulin 60 min : 29 – 88 Insulin 90 min : 26 – 84 Insulin 120 min : 22 – 79

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Toll Free No. : 18001030287

REPORT

PT Name : MR. SANJAY VAZE

Ref By : DR. PARAS DEDHIA

Reg No : HL7801829210 / HYMH/BSR/H9102

Barcode : HED012024

INV : Ionized Calcium

Age : 48 Year | Sex : Male

Registered on : 28-08-2026 12:57 PM

Received on : 28-08-2026 05:26 PM

Reported on : 28-08-2026 08:36 PM

SAMPLE COLLECTED AT :

REDCHECK LABS



SAMPLE : Serum

IONIZED CALCIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Calcium Ionized	1.20	mmol/L	1.15 - 1.33

Clinical significance :

Ionized calcium amounts for 50-55% of total calcium and is physiologically active form of calcium. Total calcium is often difficult to interpret as it is dependent on albumin & other proteins. Alterations in serum albumin during an acute illness may change the total serum calcium by as much as 30%.

1. Ionized calcium has inverse relationship with blood pH, values can change upto 5% for each 0.1pH unit change.

1. Decreased serum ionized calcium values are seen in primary hypoparathyroidism, vitamin D deficiency, systemic illnesses leading to respiratory acidosis or metabolic alkalosis, critically ill patients, renal disorders and patients with recent blood transfusions. Decreased ionized calcium levels between 0.75 and 1 mmol/L are usually well tolerated, but the risk of reduced cardiac output & arrest increases when ionized calcium levels approach 0.62 mmol/L.

Increased serum ionized calcium values are seen in primary hyperparathyroidism, vitamin D therapy, various malignancies producing parathyroid hormone

~~End of report~~

NRahate

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REPORT

PT Name : SAURABH KOTHARI

Ref By : DR. SELF

Reg No : HL3301832634 / HYMH/KDV/H6569

Barcode : HEE125362

INV : HY FITNESS SMART 3

Age : 40 Year | Sex : Male

Registered on : 02-09-2026 02:12 PM

Received on : 02-09-2026 05:15 PM

Reported on : 02-09-2026 06:18 PM

SAMPLE COLLECTED AT :

SHREE LAB



SAMPLE : Serum

IRON PROFILE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
IRON Method: FERROZINE METHOD WITHOUT DEPROTEINIZATION	96.5	ug/dl	65 - 175
TOTAL IRON BINDING CAPACITY (TIBC) Method: SPECTROPHOTOMETRIC ASSAY	443.5	ug/dl	225 - 535
TRANSFERRIN SATURATION % Method: CALCULATED	21.76	%	13 - 45

Clinical significance :

Iron is an essential trace mineral element which forms an important component of hemoglobin, metallocompounds and Vitamin A. Deficiency of iron, leads to microcytic hypochromic anemia. The toxic effects of iron are deposition of iron in various organs of the body and hemochromatosis.

Total Iron Binding capacity (TIBC) is a direct measure of the protein Transferrin which transports iron from the gut to storage sites in the bone marrow. In iron deficiency anemia, serum iron is reduced and TIBC increases.

Transferrin Saturation occurs in Idiopathic hemochromatosis and Transfusional hemosiderosis where no unsaturated iron binding capacity is available for iron mobilization. Similar condition is seen in congenital deficiency of Transferrin.

Please correlate with clinical conditions

~~End of report~~

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REPORT

PT Name : SUCHI MARFATIA
Ref By : DR. SELF
Reg No : HL6601832626 / HYMH/KDV/H7275
Barcode : HEN172678
INV : Lipase

Age : 58 Year | Sex : Female
Registered on : 02-09-2026 10:50 AM
Received on : 02-09-2026 05:14 PM
Reported on : 02-09-2026 09:00 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

LIPASE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
LIPASE	40.0	U/L	0 - 60

Method: Hydrolyze glycol

Interpretation

For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings like serum amylase. Serum Lipase is usually normal in patients with elevated serum amylase, having peptic ulcer, salivary adenitis, inflammatory bowel disease, intestinal obstruction, and macroamylasemia. Lipemic sera may interfere with results.

Clinical Significance:

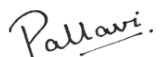
High serum Lipase is a specific marker for pancreatitis; after acute pancreatitis the Lipase activity increases within 4-8 hours, reaches a peak after 24 hours and decreases after 8 to 14 days. However, there is no correlation between the Lipase activity determined in serum and the extent of damage to the pancreas.

Please correlate with clinical conditions.

~~End of report~~



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Toll Free No. : 18001030287

REPORT

PT Name : DEEKSHA VARSHNEY (509133)

Ref By : DR. PR HOSPITAL

Reg No : HL7001832408 / MH300

Barcode : HEJ072990

INV : Luteinising Hormone (LH)

Age : 31 Year | Sex : Female

Registered on : 31-08-2026 04:45 PM

Received on : 01-09-2026 07:58 AM

Reported on : 01-09-2026 12:28 PM

SAMPLE COLLECTED AT :



SAMPLE : Serum

LUTEINISING HORMONE (LH)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE												
LUTEINISING HORMONE (LH) Method: CLIA	13.29	mIU/ml	. - . Normally menstruating												
			<table><tr><td>follicular phase</td><td>1.9–12.5</td></tr><tr><td>midcycle peak</td><td>8.7–76.3</td></tr><tr><td>luteal phase</td><td>0.5–16.9</td></tr><tr><td>Pregnant</td><td>< 0.1–1.5</td></tr><tr><td>Postmenopausal</td><td>15.9–54.0</td></tr><tr><td>Contraceptives</td><td>0.7–5.6</td></tr></table>	follicular phase	1.9–12.5	midcycle peak	8.7–76.3	luteal phase	0.5–16.9	Pregnant	< 0.1–1.5	Postmenopausal	15.9–54.0	Contraceptives	0.7–5.6
follicular phase	1.9–12.5														
midcycle peak	8.7–76.3														
luteal phase	0.5–16.9														
Pregnant	< 0.1–1.5														
Postmenopausal	15.9–54.0														
Contraceptives	0.7–5.6														

Clinical significance :

Luteinizing hormone (LH) is a glycoprotein hormone having two subunits. The alpha subunit is similar to those of follicle-stimulating hormone (FSH), human chorionic gonadotropin (hCG), and thyroid-stimulating hormone (TSH). The beta subunit is different from those of the other glycoprotein hormones and confers its biochemical specificity.^{1,2}

LH is secreted by the anterior pituitary in response to gonadotropin-releasing hormone (GnRH) secreted by the hypothalamus.^{3,4} In males, LH is also called interstitial cell-stimulating hormone (ICSH).^{1,2} In both males and females, LH secretion is regulated by a balance of positive and negative feedback mechanisms involving the hypothalamic-pituitary axis, the reproductive organs, and the pituitary and sex steroid hormones.⁵⁻⁷ LH and the other pituitary gonadotropin, FSH, play a critical role in maintaining the normal function of the male and female reproductive systems.

Abnormal LH levels with corresponding increased or decreased levels of FSH, estrogens, progesterone, and testosterone are associated with a number of pathological conditions.³ Increased LH levels are associated with menopause, primary ovarian hypofunction, and polycystic ovary disease in females and primary hypogonadism in males. Decreased LH levels are associated with primary ovarian hyperfunction in females and primary hypergonadism in males.

~~End of report~~

Moushmi Mukherjee

Dr. MOUSHMI MUKHERJEE (MD PATH)
Director (Reg. 61873)

Meenu

Dr. MEENU RANI (MD PATH)
Consultant Pathologist



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Toll Free No. : 18001030287

REPORT

PT Name : TARANNUM

Age : 48 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. YUSUF KHAN

Registered on : 31-08-2026 04:22 PM

Reg No : HL4801832378 / RAJ52

Received on : 31-08-2026 06:31 PM

Barcode : HEN112154

Reported on : 31-08-2026 06:55 PM

INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

SAMPLE : EDTA Blood



MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :
Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.
While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

NRahate

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Pallavi

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Toll Free No. : 18001030287

REPORT

PT Name : TARANNUM
Ref By : DR. YUSUF KHAN
Reg No : HL4801832378 / RAJ52
Barcode : HEN112154
INV : Malarial Falciparum And Vivax Antigen (Parasite V & F)

Age : 48 Year | Sex : Female
Registered on : 31-08-2026 04:22 PM
Received on : 31-08-2026 06:31 PM
Reported on : 31-08-2026 06:55 PM

SAMPLE COLLECTED AT :



SAMPLE : EDTA Blood

MALARIAL FALCIPARUM AND VIVAX ANTIGEN (PARASITE V & F)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
P.Vivax Method: Immunochromatography	POSITIVE	.	- - NEGATIVE
P.Falciparum Method: Immunochromatography	NEGATIVE	.	. - NEGATIVE

NOTE :
Malaria is a disease caused by a parasite. The parasite is transmitted to humans through the bites of infected mosquitoes. People who have malaria usually feel very sick, with a high fever and shaking chills. Each year, approximately 210 million people are infected with malaria, and about 440,000 people die from the disease. Most of the people who die from the disease are young children in Africa.
While the disease is uncommon in temperate climates, malaria is still common in tropical and subtropical countries. World health officials are trying to reduce the incidence of malaria by distributing bed nets to help protect people from mosquito bites as they sleep.

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : GANESH DUBAL

Ref By : DR. BALLAL

Reg No : HL2501827161 / RAJ05

Barcode : HED104491

INV : Microalbumin 24 Hrs URINE

Age : 33 Year | Sex : Male

Registered on : 26-08-2026 01:32 PM

Received on : 26-08-2026 05:14 PM

Reported on : 26-08-2026 10:03 PM

SAMPLE COLLECTED AT :



SAMPLE : Urine

MICROALBUMIN 24 HRS URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL VALUE	2000	MI/day	250 - 2400
MICRO ALBUMIN Method: PHOTOMETRIC	53.89	mg/dl	0 - 30
Microalbumin 24 Hours (URINE) Method: Spectrophotometry	1077.8	mg/dL 24 Hrs	. - .

Interpretation

-
- Thirty to 300 mg of protein is known as microalbuminuria, and it may indicate early kidney disease.
- More than 300 mg of protein is known as macroalbuminuria, and it indicates more advanced kidney disease.

Several temporary factors can cause higher-than-normal urinary microalbumin results, such as:

- blood in your urine, or hematuria
- a fever
- recent vigorous exercise
- dehydration
- a urinary tract infection

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : MR. AVTAR SINGH KATTAURA

Ref By : DR. LAVANYA

Reg No : HL8801832380 / HYMH/MH642

Barcode : HEM072694

INV : health Check Extended Male

Age : 46 Year | Sex : Male

Registered on : 31-08-2026 04:30 PM

Received on : 31-08-2026 05:39 PM

Reported on : 31-08-2026 07:32 PM

SAMPLE COLLECTED AT :

PROSPERITY DIAGNOSTICS (SAMEER)



SAMPLE : Urine

MICROALBUMIN & CREATININE RATIO (URINE)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
URINARY MICROALBUMIN Method: Turbidometry	15.19	mg/dL	0.0 - 30.0
CREATININE - URINE Method: Creatinine Jaffe Method, Rate-Blanked and Compensated	139.58	mg/dL	39.0 - 259.0
Microalbumin/ Creatinine Ratio (ACR) Method: CALCULATED	10.88	mg/g	0 - 30

Comments

Please correlate with clinical conditions.

Clinical Information .

Urine albumin to creatinine ratio (ACR), also known as urine microalbumin, helps identify kidney disease that can occur as a complication of diabetes.

If kidney disease is diagnosed early in people with [diabetes](#), appropriate treatment can be given and its effects can be closely monitored.

This means a person's ACR level should be checked as soon as diabetes is diagnosed.

It should also be measured each year, or more frequently, if your ACR level is significantly raised.

If you have a slightly raised ACR level, you may have early-stage kidney disease. A very high ACR level indicates more severe kidney disease. A very low ACR value probably means your kidneys are functioning normally.

If diabetes is well managed, it's easier to control or prevent complications developing, such as [high blood pressure](#), which can sometimes lead to kidney disease.

~~End of report~~

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Pallavi

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Toll Free No. : 18001030287

REPORT

PT Name : MRS SANGITA BAPU GAWALI

Age : 48 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. YASHRAJ LAB

Registered on : 28-08-2026 06:55 PM

BHUSHAN BANKAR

Reg No : HL1401829575 / MH004/008

Received on : 29-08-2026 01:16 AM

Barcode : HEJ088961

Reported on : 30-08-2026 01:01 PM



INV : PAP Smear

SAMPLE : SLIDE

PAP SMEAR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
------------------	--------	-------	----------------------------

SPECIMEN TYPE : CONVENTIONAL PAP'S SMEARS

SPECIMEN ADEQUACY : Satisfactory for evaluation.

COMMENTS : The smear shows clusters of intermediate and superficial cells. Doderlein bacilli seen.

IMPRESSION : NFIL/M

~~End of report~~

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REPORT

PT Name : MISS .SNEHA YADAV

Age : 11 Year | Sex : Female

SAMPLE COLLECTED AT :

Ref By : DR. DR.SAMIR KAPADIA

Registered on : 31-08-2026 10:18 AM

SAMIT DIAGNOSTIC CENTER

Reg No : HL9501831765 / MH777

Received on : 31-08-2026 04:27 PM

Barcode : HEN102616

Reported on : 01-09-2026 11:07 AM



INV : Complete Blood Count (CBC) +PS+ESR

SAMPLE : EDTA Blood

PERIPHERAL BLOOD SMEAR (PBS O R PBF)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
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RBC- Predominantly normocytic normochromic

WBC- Within normal limits

Platelets- Adequate on smear

~~End of report~~

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REPORT

PT Name : MRS. AAYESHA KHATOON
CHOUHARY

Ref By : DR. VARUN BHATIA

Reg No : HL7201826910 / MH549

Barcode : HEJ042813

INV : Potassium

Age : 50 Year | Sex : Male

Registered on : 26-08-2026 11:37 AM

Received on : 26-08-2026 04:49 PM

Reported on : 26-08-2026 05:29 PM

SAMPLE COLLECTED AT :

SUCCESS DIAGNOSTIC CENTRE



SAMPLE : Serum

POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	3.66	mEq/L	3.8 - 5.4

~~End of report~~



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☎ Toll Free No. : 18001030287

REPORT

PT Name : MRS. AAYESHA KHATOON
Ref By : DR. VARUN BHATIA
Reg No : HL7201826910 / MH549
Barcode : HEJ042813
INV : Potassium

Age : 50 Year | Sex : Male
Registered on : 26-08-2026 11:37 AM
Received on : 26-08-2026 04:49 PM
Reported on : 26-08-2026 05:29 PM

SAMPLE COLLECTED AT :
SUCCESS DIAGNOSTIC CENTRE



SAMPLE : Serum

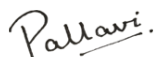
POTASSIUM

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
POTASSIUM , SERUM Method: (Electrode)	3.66	mEq/L	3.8 - 5.4

~~End of report~~



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REPORT

PT Name : MRS. NAUSHEEN SHAIKH

Ref By : DR. SELF

Reg No : HL1101814476 / MH537

Barcode : HEE152025

INV : Pregnancy Test Urine

Age : 26 Year | Sex : Female

Registered on : 15-08-2026 11:48 AM

Received on : 15-08-2026 06:20 PM

Reported on : 15-08-2026 08:45 PM

SAMPLE COLLECTED AT :



SAMPLE : Urine

PREGNANCY TEST URINE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PREGNANCY TEST, URINE Method: ICT	POSITIVE	.	. - .
INTERPRETATION:			
RESULT (INDEX)	REMARKS		
Positive	Indicates presence of HCG in urine.		
Negative	Indicates absence of HCG in urine.		

NOTE:

1. Positive results in urine are seen in pregnancy in or after 1st week of missed period.
2. Negative results in urine are seen in woman who are unlikely to be pregnant. A repeat sample to be collected a week apart to confirm the diagnosis of pregnancy.
3. False positive results may be observed in conditions like Trophoblastic disease, Non-trophoblastic neoplasms, intake of fertility drugs or other medications that contain HCG, immediately after an abortion, ectopic pregnancy, menopause or ovarian problems.
4. False negative reaction may be seen if the HCG level in urine is below the detection level of the test, sample obtained is diluted, sample collected immediately after the missed period and intake of medications like diuretics, promethazine etc.
5. Test conducted on urine.

USES:

- To establish presumptive diagnosis of pregnancy

~~End of report~~

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☎ Toll Free No. : 18001030287

REPORT

PT Name : FATIMA BI TAHIR SAYYED

Ref By : DR. NIDA ANSARI

Reg No : HL1501832477 / MH621

Barcode : HEI085199

INV : Prolactin (PRL)

Age : 26 Year | Sex : Female

Registered on : 31-08-2026 05:37 PM

Received on : 01-09-2026 05:53 PM

Reported on : 01-09-2026 09:11 PM

SAMPLE COLLECTED AT :

NEW LIFE CLINIC



SAMPLE : Serum

PROLACTIN (PRL)

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROLACTIN (PRL)	13.0	ng/ml	2.1 - 17.7

Method: CLIA

REFERENCE

Nonpregnant	2.8-29.2
Pregnant	9.7-208.5
Postmenopausal	1.8-20.3

Clinical significance :

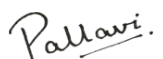
Prolactin is a single-chain polypeptide hormone secreted by the anterior pituitary¹ under the control of prolactin-inhibiting factors and prolactin-releasing factors. These inhibiting and releasing factors are secreted by the hypothalamus.² Prolactin is also synthesized by the placenta and is present in amniotic fluid. Prolactin initiates and maintains lactation in females.³ It also plays a role in regulating gonadal function in both males and females. In adults, basal circulating prolactin is present in concentrations up to 30 ng/mL (636 μ IU/mL). During pregnancy and postpartum lactation, serum prolactin can increase 10- to 20-fold. Exercise, stress, and sleep also cause transient increases in prolactin levels.

Consistently elevated serum prolactin levels greater than 30 ng/mL (636 μ IU/mL) in the absence of pregnancy and postpartum lactation are indicative of hyperprolactinemia, which is the most common hypothalamic-pituitary dysfunction encountered in clinical endocrinology. Hyperprolactinemia often results in galactorrhea, amenorrhea, and infertility in females, and in impotence and hypogonadism in males.⁴ Renal failure, hypothyroidism, and prolactin-secreting pituitary adenomas are also common causes of abnormally elevated prolactin levels.

~~End of report~~



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☎ Toll Free No. : 18001030287

REPORT

PT Name : ANIRUDDHA SATKAR

Ref By : DR. MAULI LAB

Reg No : HL5401831575 / MH244

Barcode : HEI047417

INV : Protein - Total

Age : 28 Year | Sex : Male

Registered on : 30-08-2026 08:39 PM

Received on : 31-08-2026 09:01 AM

Reported on : 31-08-2026 11:13 AM

SAMPLE COLLECTED AT :

HORMOCARE



SAMPLE : Serum

PROTEIN - TOTAL

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
PROTEIN TOTAL Method: Biruet	7.15	g/dL	6.0 - 8.0
Clinical significance :			

Please correlate with clinical conditions.

~~End of report~~



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Toll Free No. : 18001030287

REPORT

PT Name : SHIVAJI SHELAR

Ref By : DR. SELF SELF

Reg No : HL2201822408 / CH002/H9588

Barcode : HDL080575

INV : Protein Total With Albumin & PA Ratio

Age : 42 Year | Sex : Male

Registered on : 22-08-2026 11:28 AM

Received on : 22-08-2026 11:34 AM

Reported on : 22-08-2026 07:36 PM

SAMPLE COLLECTED AT :

SEVA LAB PACHORA



SAMPLE : Serum

PROTEIN TOTAL WITH ALBUMIN & PA RATIO

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
TOTAL PROTEIN Method: End point	7.32	g/dL	6.0 - 8.0
ALBUMIN - SERUM Method: PHOTOMETRY	3.87	gm/dl	3.2 - 5.2
PROTEIN/ALBUMIN RATIO Method: .	1.89	Ratio	. - .

~~End of report~~

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Toll Free No. : 18001030287

REPORT

PT Name : MR. KUNAL BORHADE

Ref By : DR. J.S.MOJITYA

Reg No : HL4901832488 / MH520

Barcode : HEI011182

INV : Prothrombin Time (PT) With INR

Age : 33 Year | Sex : Male

Registered on : 31-08-2026 05:45 PM

Received on : 31-08-2026 07:05 PM

Reported on : 31-08-2026 08:21 PM

SAMPLE COLLECTED AT :



SAMPLE : Citrated Plasma

PROTHROMBIN TIME (PT) WITH INR

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
Prothrombin Time Patient Value Method: Clotting	15.8	sec	11 - 16
Prothrombin Time Control Value Method: Clotting	13.5	sec	. - .
Prothrombin Time INR Method: CALCULATED	1.17	.	0.90 - 1.10
index Method: cu	85.44	%	---

Clinical significance :

Prothrombin time (PT) is a blood test that measures how long it takes blood to clot. A prothrombin time test can be used to check for bleeding problems. PT is also used to check whether medicine to prevent blood clots is working.

A PT test may also be called an INR test. INR (international normalized ratio) stands for a way of standardizing the results of prothrombin time tests, no matter the testing method. It lets your doctor understand results in the same way even when they come from different labs and different test methods. In some labs, only the INR is reported and the PT is not reported.

Other blood clotting tests,

such as partial thromboplastin time (PTT) and activated clotting time (aPTT), might be used if you take another type of blood-thinning medicine called heparin. These tests measure other clotting factors, or they may be used to see if you are getting the right dose of heparin. Partial thromboplastin time and prothrombin time are often done at the same time to check for bleeding problems or the chance for too much bleeding in surgery.

Blood clotting factors are needed for blood to clot (coagulation). Prothrombin, or factor II, is one of the clotting factors made by the liver. Vitamin K is needed to make prothrombin and other clotting factors. Prothrombin time is an important test because it checks to see if five different blood clotting factors (factors I, II, V, VII, and X) are present. The prothrombin time is made longer by

- Blood-thinning medicine, such as warfarin.
- Low levels of blood clotting factors.
- A change in the activity of any of the clotting factors.
- The absence of any of the clotting factors.
- Other substances, called inhibitors, that affect the clotting factors.
- An increase in the use of the clotting factors.

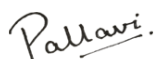
An abnormal prothrombin time is often caused by liver disease or injury or by treatment with blood thinners.

Please correlate with clinical conditions.

~~End of report~~



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